Case G — Mr. Khan

28M · Software Engineer · 2-year progressive low back & buttock pain | Working Dx: Ankylosing Spondylitis / Axial SpA

Target: 70%+ | Musculoskeletal (MSK) Physiotherapy

■ PROBLEM LIST

- Reduced lumbar flexion (Schober's test <5cm)
- Bilateral SIJ tenderness / inflammation
- Restricted chest expansion (2cm — severely reduced)
- Inflammatory pain at rest and at night
- Fatigue (common in active inflammatory disease)
- Difficulty sustaining static postures at desk (prolonged sitting as software engineer)
- Impaired sleep quality (waking 4am)
- Reduced walking tolerance during flares
- Difficulty with bending/dressing secondary to spinal stiffness

■ RED FLAGS

- Saddle Anaesthesia / Bowel-Bladder Change Significance: Cauda equina syndrome — surgical emergency Action: Immediate emergency referral to A&E; cease assessment
- Unexplained Weight Loss / Fever / Night Sweats Significance: Potential malignancy or systemic infection (e.g. spinal TB) Action: Urgent same-day GP/oncology referral; imaging required
- History of Malignancy Significance: Spinal metastases — fracture and cord compression risk Action: Urgent rheumatology/oncology referral; plain X-ray and MRI
- Acute Uveitis Flare (current) Significance: Risk of vision loss if untreated; indicates active disease flare Action: Refer to ophthalmology same day; liaise with rheumatologist regarding disease activity
- Progressive Neurological Deficit Significance: May indicate atlantoaxial instability or cord compression from fused segment fracture Action: Cease physiotherapy; urgent neurosurgical/spinal team referral

■ PATHOPHYSIOLOGY

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■ DIFFERENTIAL DIAGNOSES

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■ OBJECTIVE ASSESSMENTS & HOW TO PERFORM

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■ TREATMENT PLAN

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